

**SYNTHETIC COURT DOCUMENT — FICTIONAL CASE — NOT AN
ACTUAL FILING**

UNITED STATES DISTRICT COURT FOR THE WESTERN DISTRICT OF VIRGINIA

Civil Action No. SYN-WDVA-25-CV-0733

Deposition of Paramedic Mateo Cole

Taken September 29, 2025; designated with Ellison's December 12, 2025 opposition.

Q. State your occupation and role on February 3, 2025.

A. I am a licensed paramedic. I responded to Larkspur Court Apartments after a request to assess Mara Ellison following police use of a conducted-energy weapon. I examined her at the scene, documented findings, and participated in transport to Alder Creek Regional Medical Center.

Q. Did you witness the physical encounter?

A. No. I arrived after Ellison had been handcuffed. I did not see Officer Nolan Rusk grasp her wrist, any alleged strike or reach, the drive-stun cycle, or the takedown. My testimony concerns her condition, treatment, statements made for assessment, and the records I prepared.

Q. What information did you receive on arrival?

A. Dispatch said a person was conscious after one drive-stun application during a welfare call. At the scene, an officer reported a five-second contact cycle and no probe deployment. Ellison said she had been grabbed, shocked, and forced down. Those histories helped direct examination; they were not my independent findings about disputed movements.

Cole deposition — initial assessment

Q. Describe Ellison's presentation.

A. She was conscious, oriented, tearful, and breathing rapidly. She answered identity, location, and date questions correctly. Her pulse was elevated initially and decreased during assessment. She reported pain at the wrist, two contact sites, and one shoulder. I saw no active bleeding, loss of consciousness, or obvious deformity.

Q. Did her distress prevent an examination?

A. No. She needed pauses and reassurance, but she cooperated. I asked officers to provide space once the scene was secure. She permitted inspection of the wrist and contact sites and described pain with shoulder movement. Her distress was clinically relevant, but I did not diagnose its legal or psychiatric cause.

Q. Did she threaten anyone during your care?

A. I heard no threat and saw no attempt to strike, grab, or flee. That describes the medical-assessment period only. It does not establish what she did before I arrived. A patient may behave differently as circumstances change, so I avoid using later cooperation as conclusive proof of earlier conduct.

Cole deposition — wrist abrasion

Q. What did you observe at the wrist?

A. There was a superficial abrasion with localized redness near the wrist. Circulation, movement, and sensation were intact distally. I cleaned the area and applied a simple dressing. I saw no deep laceration, uncontrolled bleeding, or deformity suggesting a displaced fracture.

Q. Could you determine precisely how the abrasion occurred?

A. No. Its location was compatible with friction or pressure during wrist control or handcuffing, but the appearance did not identify a single mechanism. Ellison said the pain began when Rusk grasped her wrist and she recoiled. I recorded that as patient history. I did not witness the grasp and cannot use the abrasion to prove whether she pulled, struck, or reached toward equipment.

Q. Was the abrasion imaginary because it was superficial?

A. No. It was observable and warranted cleaning, even though it was not a severe open wound. Clinical severity and existence are different questions. A superficial injury can accompany a painful event. Conversely, the injury's presence does not by itself determine whether a level of force was objectively reasonable. I provide the medical facts, not a constitutional conclusion.

Cole deposition — contact marks and device history

Q. Describe the two contact marks.

A. I observed two small superficial areas consistent with contact burns. They were localized, without deep tissue injury or ongoing bleeding. Based on the reported device history, they were consistent with drive-stun contacts. I dressed them conservatively and recommended further evaluation at the medical center.

Q. Were probes removed?

A. No. I found no embedded darts, and the officers reported that no probes were fired. A drive-stun application places contacts against the body. I did not examine device data personally; my record states the mode and single five-second cycle as history supplied at the scene and later confirmed in materials shown during discovery.

Q. Can the marks establish Ellison's posture or hand position?

A. Not with the precision relevant to the parties' dispute. Their location can be described, but it does not reconstruct whether a hand struck a forearm or approached a duty belt during the obstructed interval. Nor can I infer a threat from the existence of contact marks. The marks show physical contact associated with the device, not the complete circumstances leading to its use.

Cole deposition — shoulder symptoms and imaging

Q. What shoulder complaint did Ellison make?

A. She reported pain when raising and rotating the shoulder after being forced toward the floor. There was tenderness and reduced movement because of pain, but no visible dislocation. I supported the arm during transport and advised hospital evaluation.

Q. What did the medical-center records show?

A. Imaging identified no fracture. The discharge assessment characterized the condition as a shoulder strain. The record also documented the wrist abrasion and two superficial contact burns. No surgery or inpatient admission was required. I reviewed those records to refresh my memory; the treating clinician made the final diagnosis.

Q. Does the absence of a fracture mean no meaningful force occurred?

A. No. It means imaging did not show a broken bone. Strain, abrasion, burns, and pain can occur without fracture. At the same time, I would not exaggerate these injuries into conditions not documented. The accurate description is important: wrist abrasion, two superficial contact burns, shoulder strain, and no fracture. Their legal significance is outside my expertise.

Cole deposition — patient history

Q. What account did Ellison give during assessment?

A. She said an officer gripped her wrist, she pulled because it hurt, and she did not hit the officer or reach for his belt. She said she felt a shock and was pushed down. I asked those questions to identify injury mechanisms, potential electrical exposure, and whether she had hit her head.

Q. Did she report losing consciousness or striking her head?

A. No. She denied loss of consciousness and did not report a direct head strike. Her neurological screen was reassuring. We still transported her because of the device use, shoulder complaint, distress, and need for a controlled clinical setting.

Q. Do you vouch for every disputed detail in her account?

A. No. Patient histories are recorded because they guide care. I can say she made the statements and that they were relevant to examination. I cannot verify movements I did not see. Likewise, an officer's different history helped us understand possible contact but did not become my factual finding. I separated attributed statements from objective observations in the report.

Cole deposition — mental-health context and care

Q. How did the welfare-call context affect treatment?

A. Dispatch and Ellison's sister described a medication and mental-health crisis. We reduced the number of people speaking, explained each examination step, and monitored breathing and pulse. Ellison became calmer with time and communication. I did not find an emergent medical basis to use additional physical restraint during transport.

Q. Did you diagnose Ellison as dangerous?

A. No. My assessment did not use that label. She was distressed but oriented and cooperative during care. A danger determination for an earlier moment would require facts outside my observation. I did not infer that mental illness itself created an immediate physical threat.

Q. Was the transport voluntary?

A. Ellison agreed to medical evaluation after discussion. Officers accompanied the process but did not direct me to sedate her. She entered the ambulance with assistance because of shoulder pain. I did not see a weapon in her possessions. These details describe the post-force period and should not be used as a substitute for evidence of the obscured encounter.

Cole deposition — records, custody, and limitations

Q. Did you prepare the prehospital-care report contemporaneously?

A. Yes. I began it that evening and completed it under the service's ordinary record procedure. It contains dispatch reason, times, vital signs, examination, treatment, attributed histories, transport, and transfer of care. The copy produced in discovery fairly represents my report.

Q. Did you review body-camera video when treating Ellison?

A. No. I later viewed a short segment in preparation for deposition. The central view is obstructed between 19:53:08 and 19:53:22. Nothing in my medical training allows me to infer unseen hand movements from compression, sound, or the eventual injuries. I offer no video-analysis or police-practices opinion.

Q. Could the documented injuries prove the number of device cycles?

A. The contact marks are consistent with the reported application, but device data is the better source for cycle count and duration. I was told there was one five-second drive-stun cycle. I would not calculate an additional cycle from the appearance of two contacts. Nor can the injury pattern decide whether the use was necessary. My record provides a medical boundary around the claims rather than a reconstruction.

Cole deposition — final verification

Q. Summarize the opinions you can give to a reasonable clinical certainty.

A. Ellison had an observable wrist abrasion, two superficial contact burns, and symptoms consistent with a shoulder strain after the police encounter. Imaging later showed no fracture. The findings and histories justified evaluation and conservative treatment. I cannot determine from those findings whether she struck Rusk, reached for his belt, merely pulled from pain, or posed an immediate threat.

Q. Have you kept factual observation separate from advocacy?

A. Yes. I attribute statements to their speakers, identify conditions I examined, and defer to technical witnesses for device and video data. I do not describe the body-camera obstruction as proof for either party. Medical evidence can corroborate that physical force and injury occurred without deciding every disputed circumstance.

Q. Is there any correction to your transcript designation?

A. One early note called both contact sites “punctures.” They were superficial contact marks, not embedded-probe wounds, and this transcript uses the more accurate description. The fixed injury summary is a wrist abrasion, two superficial contact burns, shoulder strain, and no fracture. I make no opinion about qualified immunity, collateral-order jurisdiction, credibility, or the ultimate reasonableness of force.